This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Phenoxymethylpenicillin 500mg tablets or Clarithromycin 250mg tablets for the treatment of Sore Throat Test & Treat

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Sore Throat Test & Treat

Acute Sore Throat Assessment

- **Aetiology:** Most commonly viral (70-80%); Group A Streptococcus (GAS) responsible for 10-15% of bacterial sore throats.
- **Clinical presentation:** Sore throat, difficulty swallowing, pharyngitis, possible tonsillitis (exudate, erythema), fever.
- **Red flags:** Trismus, drooling, dysphagia, stridor, respiratory distress - refer for emergency assessment.

FeverPAIN and Centor Score

- **FeverPAIN score:** Fever, Purulence, Attend rapidly, No cough, Ill appearance. Score 0-5; score ≥4 suggests GAS infection.
- **Centor score:** Exudate, Fever, Nodes, No cough. Score 0-4; score ≥3 or rapid antigen test positive suggests GAS.

- **NICE guidance:** Offer antibiotics if FeverPAIN ≥4 or positive rapid GAS antigen test (RAST). Do not offer antibiotics for FeverPAIN 0-3 without test.

Group A Streptococcus Testing

- **RAST (rapid antigen test): can be used in primary care or pharmacy to guide antibiotic use. A positive result is an inclusion criterion in both arms of this PGD; the test itself is not supplied under it.**
- **Throat culture:** Gold standard but results delayed; not used for acute management.
- **Management:** Use FeverPAIN score or RAST to determine need for antibiotics.

Antibiotic Treatment

- **First-line (FeverPAIN ≥4 or RAST+): Phenoxymethylpenicillin 500mg QDS for 5 days.**
- **Penicillin allergy:** Use clarithromycin 250mg BD for 5 days.
- **No treatment:** FeverPAIN 0-3 without positive RAST - advise supportive care.

Patient Group Direction

for the supply of

Phenoxymethylpenicillin (Penicillin V) 500mg tablets

for the treatment of

Sore Throat Test & Treat

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training

	was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of acute sore throat caused by Group A Streptococcus (GAS) when FeverPAIN score ≥4 or positive RAST
Inclusion criteria	Adults aged 18 years and over Acute sore throat with FeverPAIN score ≥4 OR positive rapid antigen test for Group A Streptococcus Able to take oral medication No recent antibiotic use for this illness Capable of giving valid informed consent
Exclusion criteria	Known penicillin or beta-lactam allergy (use clarithromycin instead) Stridor, difficulty breathing, drooling, inability to swallow saliva, trismus, a muffled 'hot potato' voice, unilateral peritonsillar swelling or deviation of the uvula (suspected quinsy or epiglottitis): emergency referral, 999 or A&E. Do not examine the throat with a tongue depressor where epiglottitis is possible. Signs of sepsis or systemic illness: temperature 38°C or above together with any one of heart rate above 90, respiratory rate 20 or above or systolic blood pressure below 100; or new confusion; or a patient who looks unwell, whatever the temperature: emergency referral. Immunosuppression, or a medicine that can cause neutropenia (chemotherapy, carbimazole, clozapine, methotrexate or other DMARDs): refer for a same-day full blood count and clinical assessment. Symptoms for more than 2 weeks, or a persistent unilateral neck lump, unilateral tonsillar enlargement or hoarseness for more than 3

	weeks: refer to the GP (possible malignancy pathway). Severe hepatic or renal dysfunction
Cautions	Mild to moderate renal impairment: generally safe; monitor renal function Hepatic impairment: generally safe but monitor liver function Pregnancy and breastfeeding: penicillin V is generally safe; ensure informed consent Drug interactions: may reduce efficacy of oral contraceptives; advise additional contraception for 7 days Mononucleosis: avoid due to risk of rash (if uncertain of diagnosis, clarify before prescribing)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Phenoxymethylpenicillin 500mg tablets
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from light and moisture.
Route/method of administration	Oral
Dose and frequency	500mg (one tablet) four times daily To be taken on an empty stomach (1 hour before or 2 hours after meals) for optimal absorption Continue for 5 days
Quantity to be administered and/or supplied	20 tablets (5-day course)
Maximum or minimum treatment period	5 days
Adverse effects	Very common: nausea, diarrhoea Common: rash (non-allergic), vomiting, abdominal pain Uncommon: indigestion, oral candidiasis Rare: hypersensitivity reactions (urticaria, Stevens-Johnson

	syndrome), angioedema, anaphylaxis, C. difficile infection

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form and route, and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Complete the full course of antibiotics as prescribed, even if symptoms improve within 2-3 days. Take phenoxymethylpenicillin (Penicillin V) on an empty stomach (1 hour before or 2 hours after food) for best absorption. Take clarithromycin with or without food; if GI upset occurs, take with food. If using oral contraception, use additional contraceptive methods during the antibiotic course and for 7 days afterwards. For pain relief, use paracetamol or ibuprofen as directed; a sore throat should improve within 3-5 days. Stay hydrated: drink plenty of water and other fluids to help soothe the throat. Use throat lozenges or warm salt water gargles for comfort (not part

	of medical treatment). Seek medical advice if symptoms worsen or do not improve after 3-5 days of treatment. Report any allergic reactions (rash, facial swelling, breathing difficulties) immediately. Inform your doctor if you develop persistent diarrhoea during or after clarithromycin (possible C. difficile infection). If you have a metallic taste with clarithromycin, this should resolve after stopping the medication. Do not share antibiotics with others; this course is prescribed for you alone.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Sore throat (acute): prescribing antibiotics
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	1st November 2025

Patient Group Direction

for the supply of

Clarithromycin 250mg tablets

for the treatment of

Sore Throat Test & Treat

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products

	• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of acute sore throat caused by Group A Streptococcus in patients with penicillin allergy (when FeverPAIN ≥4 or RAST positive)
Inclusion criteria	Adults aged 18 years and over Acute sore throat with FeverPAIN score ≥4 OR positive RAST Documented penicillin allergy or intolerance of any severity, including a history of anaphylaxis to penicillin Able to take oral medication No contraindications to macrolide antibiotics Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to macrolides (clarithromycin, erythromycin, azithromycin) Concurrent use of ergotamine, dihydroergotamine (risk of ergot toxicity) Concurrent use of certain statins (simvastatin, lovastatin) - increased statin levels QT prolongation or risk factors (hypokalaemia, hypomagnesaemia, cardiac arrhythmia history) Concurrent use of colchicine, ticagrelor, ranolazine, ivabradine, domperidone, pimozide, astemizole, cisapride, terfenadine, oral midazolam or lomitapide (SmPC contraindications) Severe hepatic failure in combination with renal impairment Severe hepatic impairment Myasthenia gravis (macrolides can worsen muscle weakness) Stridor, difficulty breathing, drooling, inability to swallow saliva, trismus, a muffled 'hot potato' voice, unilateral peritonsillar swelling or deviation of the uvula (suspected quinsy or epiglottitis): emergency referral, 999 or A&E. Do not examine the throat with a tongue depressor where epiglottitis is possible. Signs of sepsis or systemic illness: temperature 38°C or above together with any one of heart rate above 90, respiratory rate 20 or above or systolic blood pressure below 100; or new confusion; or a patient who looks unwell, whatever the temperature: emergency referral. Immunosuppression, or a medicine that can cause neutropenia (chemotherapy, carbimazole, clozapine, methotrexate or other DMARDs): refer for a same-day full blood count and clinical assessment.

	Symptoms for more than 2 weeks, or a persistent unilateral neck lump, unilateral tonsillar enlargement or hoarseness for more than 3 weeks: refer to the GP (possible malignancy pathway).
Cautions	Hepatic impairment: mild-moderate use with caution; avoid if severe Renal impairment (eGFR <30 mL/min/1.73m2): dose adjustment or alternative needed QT interval risk: assess baseline risk; avoid in high-risk patients Drug interactions: check every current medicine against the clarithromycin SmPC before supply. The contraindicated combinations listed under exclusion criteria exclude; they are not cautions. Pregnancy and breastfeeding: relatively safe but ensure informed consent; small amount passes to breast milk Anticoagulation: possible increased anticoagulant effect; monitor INR if on warfarin
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Clarithromycin 250mg tablets
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from moisture.
Route/method of administration	Oral
Dose and frequency	250mg (one tablet) twice daily May be taken with or without food Continue for 5 days
Quantity to be administered and/or supplied	10 tablets (5-day course)
Maximum or minimum treatment period	5 days
Adverse effects	Very common: GI upset (diarrhoea, nausea, abdominal pain), taste disturbance (metallic taste) Common: vomiting, indigestion, headache, abdominal pain, oral

	candidiasis Uncommon: hepatitis, jaundice, hypersensitivity reactions, dizziness Rare: QT prolongation, arrhythmias, Stevens-Johnson syndrome, C. difficile infection, severe hepatotoxicity

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form and route, and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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	throat should improve within 3-5 days. Stay hydrated: drink plenty of water and other fluids to help soothe the throat. Use throat lozenges or warm salt water gargles for comfort (not part of medical treatment). Seek medical advice if symptoms worsen or do not improve after 3-5 days of treatment. Report any allergic reactions (rash, facial swelling, breathing difficulties) immediately. Inform your doctor if you develop persistent diarrhoea during or after clarithromycin (possible C. difficile infection). If you have a metallic taste with clarithromycin, this should resolve after stopping the medication. Do not share antibiotics with others; this course is prescribed for you alone.

Key references

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Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	1st November 2025

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 34: phenoxymethylpenicillin course fixed at 5 days, 20 tablets, as the one quantity for the arm; the 5 to 10 day range and the 20 to 40 tablet range are removed from the guidance summary, dose row, quantity row and treatment period.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover and arm titles corrected from administration to supply.

Sepsis exclusion reworded in both arms so that fever with any one marker, new confusion, or a patient who looks unwell each trigger emergency referral.

Clarithromycin arm: the myasthenia gravis caution removed because myasthenia gravis is an exclusion in the same arm.

Clarithromycin arm inclusion no longer says non-anaphylaxis is preferred for macrolide use; penicillin allergy of any severity, including anaphylaxis, qualifies.

Guidance summary now says a positive RAST is an inclusion criterion under this PGD (it said RAST was not part of the PGD).

Records row template text separated into date, dose, form, route and quantity, and the adverse reaction bullet no longer runs into 'supplied via PGD'.

Change history tables now point to the version and change record for versions after 001.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Red-flag exclusions added to both arms: airway compromise and suspected quinsy or epiglottitis (999), sepsis and systemic illness (emergency referral), immunosuppression and neutropenia-risk medicines (same-day FBC), and symptoms over 2 weeks or a persistent unilateral neck lump (GP, malignancy pathway). The previous version's only exclusions were allergy, a muddled pregnancy line and severe organ dysfunction. The pregnancy line is removed: phenoxymethylpenicillin is not contraindicated in pregnancy.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body

of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

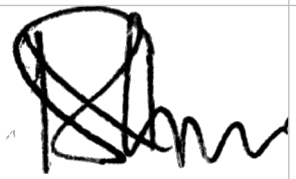
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Clarithromycin arm, exclusion criteria: the SmPC (4.3) contraindicates clarithromycin with colchicine, ticagrelor, ranolazine, ivabradine, domperidone, pimozide, astemizole, cisapride, terfenadine, oral midazolam and lomitapide, and in severe hepatic failure combined with renal impairment. None was an exclusion. All added, in the words of the SmPC.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.